February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy, National Coordinator for Health Information Technology
Department of Health & Human Services
200 Independence Ave, SW
Washington, DC 20201

RE: Request for Information: HHS Health Sector AI RFI (RIN 0955-AA13)

Dear Dr. Keane:

Thank you for the opportunity to respond to the HHS Health Sector AI RFI. We appreciate your ongoing commitment to health tech modernization and expanding access to high-quality virtual care services. As you know, virtual care has long been leveraged by health care providers across the country to enhance patient experiences, improve health outcomes, and reduce costs. Now, more than ever, virtual care is providing Americans with access to critical healthcare services, and we encourage the Department to continue to lead with focused policy that supports the appropriate expansion of artificial intelligence and digital health tools across the ecosystem.

Teladoc Health is the global leader in virtual care. The company is delivering and orchestrating care across patients, care providers, platforms and partners—transforming virtual care into a catalyst for how better health happens. Through its relationships with health plans, employers, and health systems, Teladoc Health fuels clinical excellence and applies the power of technology to help people live their healthiest lives. Notably, more than 100 million people have access to a Teladoc Health product or service.

Severe provider shortages persist across the country. For many people, the question isn't where they would get care – it's whether they get care at all. As an example, the expansion of mental health access is a top priority for Secretary Kennedy and the Trump Administration, and the workforce data demonstrates that solving this specific public policy challenge will require new technology and innovative care models to address the underlying issue.

Specifically, the Bureau of Health Workforce under the Health Resources and Services Administration (HRSA) reports that more than 137 million Americans live in a mental health care provider shortage area with only 27.3% of total need met.[1] Outside of the Northeast, most counties in every state qualify as shortage areas.[2] Notably, the data does not account for long wait times and other barriers to patients accessing care. For instance, the American Psychologist Association reports that 60% of their members were "reporting no openings for new patients" with more than 40% of their members carrying "waiting lists of 10 or more patients."[3] Even more concerning, nearly half of the membership reported burn out, indicating that they may be retiring or cutting back on workload in the coming years – thus exacerbating

[1] https://www.kff.org/other-health/state-indicator/mental-health-care-health-professional-shortage-areas-hpsas/?currentTimeframe=0&sortModel=%7B%22colId%22:%22Location%22,%22sort%22:%22asc%22%7D
[2] https://www.ruralhealthinfo.org/charts/7
[3] https://www.apa.org/monitor/2023/04/mental-health-services-wait-times

the already persistent problem. This dire warning is confirmed by HRSA, which reports that by 2038 there will be shortages of roughly 100,000 counselors, 100,000 psychologists, 77,000 addiction counselors, 33,000 marriage and family therapists, and 43,000 psychiatrists – resulting in a total shortage of over 353,000 providers.[4] To compound matters, these numbers "are based on current use of behavioral health services" and "do not address the large amount of unmet need that exists for these same services."[5] For example, an "additional 136,325 psychologists would be required by 2038 to meet all unmet need for psychologists."[6]

The American healthcare system continues to struggle to deliver critical services. There simply are not enough providers to match demand. To address these critical needs, the Administration should consider all possible policy options to extend access to care, make providers more effective and efficient, and embrace new, innovative care delivery models which allow patients who have traditionally been overlooked by the healthcare industry to receive needed care – often for the first time in their lives. Importantly, HHS and the healthcare sector must partner to rapidly advance AI tools and technology to safely scale access to care.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

States are increasingly acting to address what they perceive as a regulatory void surrounding the use of artificial intelligence in health care. In 2025, state legislatures introduced 1,208 AI-related bills, 145 of which were enacted into law. In 2026, states have already introduced 1,183 additional bills across 46 jurisdictions. To date, more than 20 states have introduced legislation specifically targeting the use of AI in therapy services. Many of these proposals are modeled on Illinois' 2025 statute, HB1806, which has drawn significant criticism for its broad scope, drafting deficiencies, and restrictive operational requirements.

IL HB1806, for example, defines "therapy services" in an overly expansive manner—encompassing any activity that could "improve" an individual's mental health—and contains notable oversights, including the exclusion of physicians as eligible mental health providers. The law also restricts the use of AI systems for any activity classified as a "therapeutic communication" or for functions involving the detection of emotions or mental states. As a result, several organizations have already indicated that they will not deploy AI-enabled tools in Illinois due to these constraints.

The accelerating and highly fragmented state legislative activity makes it difficult for healthcare technology developers to scale AI solutions nationally. States are adopting divergent requirements related to disclosure, standards of care, bias mitigation, reporting, and other compliance obligations.

As a result, providers face substantial uncertainty and operational risk when attempting to plan for the development, deployment, and long-term support of AI systems in clinical settings. For organizations operating in multiple states, integrating AI into clinical workflows or patient-facing services requires significant investment and cannot be easily reversed. Providers and developers are hesitant to proceed without confidence that the states in which they operate will not enact new restrictions in the next

---

[4] https://bhw.hrsa.gov/data-research/projecting-health-workforce-supply-demand
[5] Id.
[6] Id.

legislative session. Given the nature of most AI systems and their ability to scale rapidly across markets, this evolving patchwork presents a real and immediate barrier to innovation.

A similar challenge has already emerged in the context of state data privacy laws. Today, 20 separate state privacy regimes impose inconsistent and often conflicting obligations on the private sector, with estimates suggesting that this fragmentation could cost U.S. companies more than $1 trillion over the next decade. Small businesses report average annual compliance costs of $50,000. Without federal action, AI-related requirements may follow a comparable trajectory, compounding the regulatory burden on the healthcare sector.

For these reasons, we support continued federal leadership on AI. The Department could pursue several approaches to establish a unified national framework for the development, validation, and deployment of AI in healthcare. One option is the creation of a **Health Care Artificial Intelligence Sandbox Program**, through which AI systems would be tested, validated, and registered with the appropriate federal health agency. This approach could inform and shape future comprehensive federal legislation needed to regulate the use of AI in healthcare, which should include explicit pre-emption of existing and future state laws, similar to the structure established under the Health Insurance Portability and Accountability Act (HIPAA).

Regardless of the mechanism, it is essential that the federal government address the current regulatory gray area at the critical intersection of AI and clinical care. Without a clear and consistent federal framework, the proliferation of conflicting state laws will continue to impede innovation, limit scale, and create substantial uncertainty for providers and innovators seeking to responsibly integrate AI into clinical care.

If additional states enact similar legislation, the cumulative effect could significantly impede the ability of developers, deployers, providers, and patients to access clinically effective AI-driven systems and tools. The emergence of a fragmented, state-by-state regulatory regime, particularly one modeled on a law as restrictive as IL HB1806, would create substantial barriers to innovation, limit scalability, and constrain the responsible integration of AI technologies into mental health and broader clinical care.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

As stated above, a patchwork of state laws will exponentially increase the legal and regulatory challenges facing developers and deployers. As we discuss the novel legal and implementation issues structures below, it should be noted that these issues would likely be present in every state – thus increasing the complexities by a magnitude of 50.

Liability and indemnification

Health care organizations deploying artificial intelligence systems continue to face significant uncertainty with respect to liability and medical malpractice. Providers assessing AI in the clinical environment face many questions such as:

- Who is responsible when an AI system inaccurately diagnoses a patient and that patient is harmed? Who would that patient seek damages from?
- Which applications of AI in which clinical scenario are supported by the standard of care?
- Does medical malpractice insurance indemnify a provider when relying on an AI system's output?

As cases are decided by the courts, some questions will be answered; however, it is critical that patients and provider organizations have a clear pathway to address potential harms. The Department can play an important role and defining appropriate uses of AI and facilitating the development of clinical guidelines and pre- and post-deployment standards.

Privacy and Security

Artificial intelligence systems that support or augment clinical care inevitably interact with sensitive patient information. Whether used for diagnosis, clinical decision support, care coordination, or patient-facing services, these systems process data that is functionally indistinguishable from the information handled by traditional covered entities. AI systems routinely access, generate, or infer protected health information (PHI). Ensuring HIPAA-level protections is therefore necessary to maintain patient confidentiality, prevent misuse, and reduce the risk of data breaches. For this reason, AI developers and deployers should be held to the same privacy and security standards established under the Health Insurance Portability and Accountability Act (HIPAA).

HIPAA establishes baseline obligations, such as minimum necessary use, data integrity controls, breach notification requirements, and safeguards against unauthorized access, that are essential for maintaining trust in digital health technologies. As AI tools become more deeply embedded in clinical workflows, the absence of consistent privacy standards would create arbitrary distinctions between traditional providers and AI-driven services, undermining patient protections.

Additionally, patients reasonably expect that any entity handling their health information, whether human or algorithmic, will adhere to the same protections. Without clear federal requirements, consumers face uneven and unpredictable safeguards depending on which entity handles their data, the type of system used, or the state in which care occurs. Applying HIPAA to AI deployers ensures a uniform and comprehensible privacy framework that aligns with patient expectations and longstanding federal privacy principles.

Finally, holding AI systems to HIPAA standards creates regulatory clarity for developers. The current landscape of divergent state laws already imposes substantial compliance burdens on innovators. A federal requirement that AI systems used for medical purposes meet HIPAA standards would create a consistent national baseline, reduce fragmentation, and facilitate responsible AI deployment across jurisdictions.

In terms of security, AI systems should embed cybersecurity protections at every stage of the AI lifecycle, including model development, training, deployment, and monitoring. The Health Sector Coordinating Council (HSCC) identifies secure-by-design as a core workstream for AI in healthcare, emphasizing cross-disciplinary collaboration between engineering, cybersecurity, regulatory, and clinical teams. This includes implementing hardened defaults for authentication, logging, and data access as well as ensuring adversarial robustness to precent manipulation of model inputs or outputs.

Further, CISA's 2025 AI Data Security guidance underscores that the accuracy, integrity, and trustworthiness of AI outcomes depend on safeguarding the data that trains and operates these systems. This principle is especially critical in healthcare, where compromised data could lead to misdiagnosis, harmful clinical recommendations, or identity exposure. We believe it is imperative that systems are constantly monitored throughout their lifetimes to ensure that they remain compliant and accurate.

5. How can HHS best support private sector activities (*e.g.,* accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

To drive innovation, we believe developers of AI systems need a robust, risk-based framework to address the complex legal and regulatory issues noted above, provide clarity on what is required from the developer while simultaneously creating a robust patient safety framework.

Specifically, HHS should consider the steps needed to move toward a foundational regulatory framework for AI applications in health care that ensure responsible development, transparency, post-market oversight, and robust risk management. We remain deeply concerned about the emerging patchwork of conflicting state laws, which can only be addressed by a comprehensive federal approach. To advance the development of a federal health AI framework, we recommend that HHS:

- Establish a health care AI regulatory sandbox – in line with President Trump's 2025 AI Action Plan – through which AI systems would be tested, validated, and registered with the appropriate federal health agency.
- Invest in the development of consensus-based standards that can evolve rapidly and responsively alongside advancements in AI technology and ensure any federal framework remains nimble.

Thank you for the opportunity to provide feedback on these important issues. If you have any questions or would like to further discuss our comments, please contact Kevin Harper, Vice President, Government Affairs at kevin.harper@teladochealth.com

Sincerely,

Kevin Harper
Vice President & Head of Government Affairs
Teladoc Health